

OPERATIONS & COMPLIANCE

Infection Control, Sterilization & Sharps

Hand hygiene, instrument reprocessing, spore testing, sharps handling and exposure response

DOCUMENT OPS-004	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol covers the infection prevention program: hand hygiene, personal protective equipment, treatment room cleaning, instrument reprocessing with biological monitoring, single-use device policy, sharps and medical waste handling, and the response to a bloodborne pathogen exposure.

A DIFFERENT REGULATOR

Much of this sits under occupational safety and public health authority rather than your medical board — different inspectors, different penalties, and obligations that apply to you as an employer regardless of your clinical licensure. Most practices own the autoclave and not the logbook.

2 Hand Hygiene & PPE

Procedure category	Minimum requirement
All patient contact	Hand hygiene before and after every patient encounter, and after glove removal. Alcohol-based rub unless hands are visibly soiled, in which case soap and water.
Injections and skin penetration	Hand hygiene, clean gloves, skin antisepsis at the site, and a clean field. Gloves are changed between patients without exception.
Procedures with splash or spray risk	Add eye protection and a mask. This includes any device generating plume or aerosol.
Instrument reprocessing	Heavy-duty utility gloves, eye protection, mask and a fluid-resistant gown or apron during cleaning.
Plume-generating device use	Smoke evacuation at source plus respiratory protection appropriate to the device — see the laser safety protocol in the Skin & Laser kit.

NAILS, JEWELRY AND SLEEVES

Artificial nails, chipped polish, rings with stones and wrist jewelry all interfere with effective hand hygiene. Set a written standard rather than relying on individual judgement, and apply it to everyone including owners and visiting injectors.

3 Instrument Reprocessing

- 1 **Point-of-use cleaning.** Remove gross soil immediately; do not allow bioburden to dry onto instruments.
- 2 **Cleaning.** Manual or ultrasonic cleaning with an enzymatic detergent. Inadequate cleaning invalidates every step that follows — sterilization does not compensate for it.
- 3 **Inspection and drying.** Inspect for residue and damage. Dry fully before packaging.
- 4 **Packaging.** Appropriate pouches or wrap with a chemical indicator inside each pack and an external process indicator. Label with the date and load identifier.
- 5 **Sterilization.** Run the cycle appropriate to the instrument and packaging. Do not overload the chamber.
- 6 **Cycle verification.** Confirm the cycle completed with correct parameters and the chemical indicator changed. A pack with a failed indicator is not used.
- 7 **Storage.** Store packs clean, dry and protected. Any pack that is wet, torn or dropped is reprocessed.
- 8 **Logging.** Record every load: date, contents, parameters, operator and indicator result.

Biological monitoring

- Spore test run at least weekly, and after every autoclave repair or cycle failure.
- Results recorded and retained with the cycle logs.
- A positive spore test triggers the recall procedure below and the autoclave is removed from service until a repeat test is negative.
- Autoclave serviced and calibrated on the manufacturer's schedule, with records retained.

POSITIVE SPORE TEST — RECALL PROCEDURE

Take the autoclave out of service. Quarantine every pack processed since the last negative test. Identify every patient treated with instruments from those loads. Notify the medical director immediately. Reprocess all quarantined instruments after the unit is repaired and re-tested. The medical director decides on patient notification, and that decision — either way — is documented with its reasoning.

4 Single-Use Devices & Sharps

- ☐ Single-use devices are used once and discarded. Needles, cannulas, microneedling cartridges and disposable tips are never reprocessed, sharpened or shared.
- ☐ Single-dose vials are used for one patient and discarded, even if product remains.
- ☐ Multi-dose vials, where used at all, are dedicated to one patient where possible, dated on opening, and discarded per the beyond-use date.
- ☐ Needles are never recapped by hand; a one-handed technique or a safety device is used where recapping is unavoidable.
- ☐ Sharps containers are puncture-resistant, mounted at point of use, and replaced at the fill line rather than when full.

- ☐ Medical waste segregated, stored securely, and collected by a licensed contractor with manifests retained.
- ☐ Sharps container disposal and waste manifests logged and retained per state requirements.

THE REUSE TEMPTATION

Reprocessing a single-use microneedling cartridge or a filler cannula is a bloodborne pathogen transmission risk, a device labeling violation, and effectively indefensible if it ever comes to light. Cost pressure is not a mitigating circumstance.

5 Exposure Response

- 1 **Immediate first aid.** Wash the site with soap and water. Flush mucous membranes or eyes with water or saline for several minutes. Do not squeeze or scrub the wound.
 - 2 **Report immediately.** To the supervisor and the medical director — the same shift, not the next day. Time matters for post-exposure prophylaxis.
 - 3 **Seek medical evaluation urgently.** Occupational health or emergency department. Post-exposure prophylaxis is time-sensitive and delay reduces its effectiveness.
 - 4 **Document the exposure.** Date, time, location, device involved, circumstances, depth and type of exposure, and PPE in use at the time.
 - 5 **Source patient information** handled in accordance with applicable law and consent requirements. Do not test or disclose outside that framework.
 - 6 **Follow-up testing and counseling** per the evaluating clinician's schedule, tracked to completion.
 - 7 **Investigate and correct.** Identify what allowed the exposure and change it — the device, the technique, the container placement, or the workflow.
- Hepatitis B vaccination offered to all staff with occupational exposure risk, with acceptance or declination documented.
 - Exposure control plan reviewed annually and after any exposure incident.
 - Exposure records retained confidentially and separately from the general personnel file.

6 Environmental Cleaning & Logs

Frequency	Task	Logged
Between patients	Treatment surfaces, chair, headrest and equipment contact points disinfected with an appropriate product at the correct contact time.	No log; observed in audit
Daily	Full room clean, floors, waste removal, replenishment of PPE and supplies.	Daily checklist
Weekly	Spore test; deep clean; expiry sweep of clinical consumables.	Spore log; expiry log
Monthly	Sharps container audit; PPE stock; disinfectant contact-time verification.	Monthly checklist
Annually	Exposure control plan review; infection control training for all staff; autoclave service.	Training and service records

CONTACT TIME IS THE COMMON FAILURE

Surface disinfectants require a specified wet contact time to be effective. Wiping a surface and immediately drying it does not disinfect it. Confirm the contact time for the product you actually use and train to it.

7 Quick Reference

QUICK REFERENCE CARD

Infection control non-negotiables

1. **Hand hygiene** before and after every patient, and after gloves.
2. **Clean before you sterilize.** Bioburden defeats the cycle.
3. **Indicator in every pack**, log every load.
4. **Spore test weekly** and after any repair. Positive → recall procedure.
5. **Single-use means once.** No exceptions, no cost argument.
6. **Never recap by hand.** Sharps container at point of use.
7. **Exposure → wash, report same shift, seek care urgently.**
8. **Respect contact time** on surface disinfectants.

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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